

Breath Support — Practice Sheet

Aaron · built from the measured analysis of **Pressure Down** (Captain Cook Tavern, 25 Jul 2026). Exercise text is copied verbatim from the engine's prescription library.

Primary limiter	Breath support — severity 49/100
Evidence	49% of phrase endings sag (25 of 51 phrases)
What it sounds like	Notes arrive on pitch, hold about a second, then slide down. Sliding, not breaking.
Where it bites	Worst from 2:44 onward — 2:44 (-1.9 semitones), 3:01 (-2.2), 3:25 (-0.8). Early in the song you slide <i>up</i> into notes instead.
Your cue	“Keep the ribs wide and let the air meter out slowly.”

A 10-minute session

1	Straw in water	2 min	warm up, get the voice easy
2	Rib Cage Stationary Drill	3 min	names your exact fault: ribs must not collapse
3	Sibilant Hiss	2 min	closest to singing — air actually flowing
4	The 3:01 phrase	3 min	one phrase only, ribs staying wide

Every exercise in this library carries the same transfer rule: **apply the sensation to one short phrase only — do not run the whole song until the cue holds.**

The eight breath-support exercises

1. The Farinelli Maneuver

Target: Suspended Breath Control. · **The one your analysis prescribed first.**

- Do this

Inhale for 4 counts, suspend (hold with open glottis) for 4, exhale for 4; increase to 10+ counts.
- Should feel

Inhale low and wide, suspend without closing the throat, then release like a slow leak.
- Pass / fail

Counts are even and the throat stays open during the suspension.

Stop or reduce intensity if dizziness, shoulder lifting, throat gripping, or breath-holding appears.

2. Rib Cage Stationary Drill

Target: Thoracic Stability. · **Most direct hit on your fault.**

- Do this

Place hands on lower ribs; inhale/exhale while keeping the ribs expanded and the sternum high.
- Should feel

Keep the ribs gently open as if the body is staying wide around the sound.
- Pass / fail

Ribs do not collapse at the start of the exhale.

Stop or reduce intensity if dizziness, shoulder lifting, throat gripping, or breath-holding appears.

3. The Sibilant Hiss

Target: Steady Airflow Management. · **Closest to real singing.**

- Do this

Inhale and release a constant [s] sound for as long as possible, aiming for 30-60 seconds.
- Should feel

Let the [s] feel like one thin, steady thread of air.
- Pass / fail

The hiss stays even with no sudden bursts or fading.

Stop or reduce intensity if dizziness, shoulder lifting, throat gripping, or breath-holding appears.

4. Pulsated Fricatives

Target: Abdominal Flexibility.

Do this	Rapid, rhythmic bursts of [f] or [v] sounds (e.g., in triplets) to engage the abdominal wall.
Should feel	Let the belly wall bounce lightly without locking the throat.
Pass / fail	Pulses are even and quick without neck tension.

Stop or reduce intensity if dizziness, shoulder lifting, throat gripping, or breath-holding appears.

5. The Tissue Flutter

Target: Visual Flow Biofeedback.

Do this	Sustaining a vowel while blowing against a single tissue; a steady angle indicates consistent air pressure.
Should feel	Keep the tissue angle steady; do not blast it.
Pass / fail	Tissue movement stays consistent across the vowel.

Stop or reduce intensity if dizziness, shoulder lifting, throat gripping, or breath-holding appears.

6. Back-Breathing Expansion

Target: Posterior Lobe Activation.

Do this	Place hands on the lower back/kidney area and breathe until expansion is felt against the thumbs.
Should feel	Feel the lower ribs widen and stay buoyant; throat quiet; shoulders heavy; airflow steady rather than dumped.
Pass / fail	Airflow stays even for the target count and the body remains expanded without visible neck effort.

Stop or reduce intensity if dizziness, shoulder lifting, throat gripping, or breath-holding appears.

7. Silent Inhalation Drill

Target: Laryngeal Abduction.

Do this	Rapidly inhaling through an open throat without making noise to ensure full glottal opening.
Should feel	Breathe as if surprised, but silent — cool air, open back of throat.
Pass / fail	No audible gasp or shoulder lift.

Stop or reduce intensity if dizziness, shoulder lifting, throat gripping, or breath-holding appears.

8. Axial Alignment Scan

Target: Postural Equilibrium.

Do this	Standing with feet shoulder-width, knees unlocked, and weight evenly distributed for optimal airflow.
Should feel	Feel the lower ribs widen and stay buoyant; throat quiet; shoulders heavy; airflow steady rather than dumped.
Pass / fail	Airflow stays even for the target count and the body remains expanded without visible neck effort.

Stop or reduce intensity if dizziness, shoulder lifting, throat gripping, or breath-holding appears.

Safety. Every exercise here shares one stop rule: dizziness, shoulder lifting, throat gripping or breath-holding means stop or ease off. This sheet relays the library your analysis prescribes from and is not a substitute for a teacher watching you do it.